UPON DISCOVERING A CASUALTY

ALERT YOUR SHIPMATES

- "CASUALTY, CASUALTY, CASUALTY. Casualty in…(what space)"
- If hazmat, yell "HAZMAT, HAZMAT, HAZMAT. Hazmat in …" first. Then yell "CASUALTY…" to limit how many people enter the hazmat hot zone

SCENE SURVEY

- Ensure the scene is safe
  - Check for **fire / wire / gas / glass**
  - Don't become another casualty!
- What is the mechanism of injury (MOI)?
- How many casualties are there?
- Send for **equipment + medical aid**

APPROACHING THE CASUALTY

- Ensure you have proper PPE before touching the casualty
  - gloves, pocket mask, face shield
- Identify yourself, tell casualty not to move
- Take charge of the casualty

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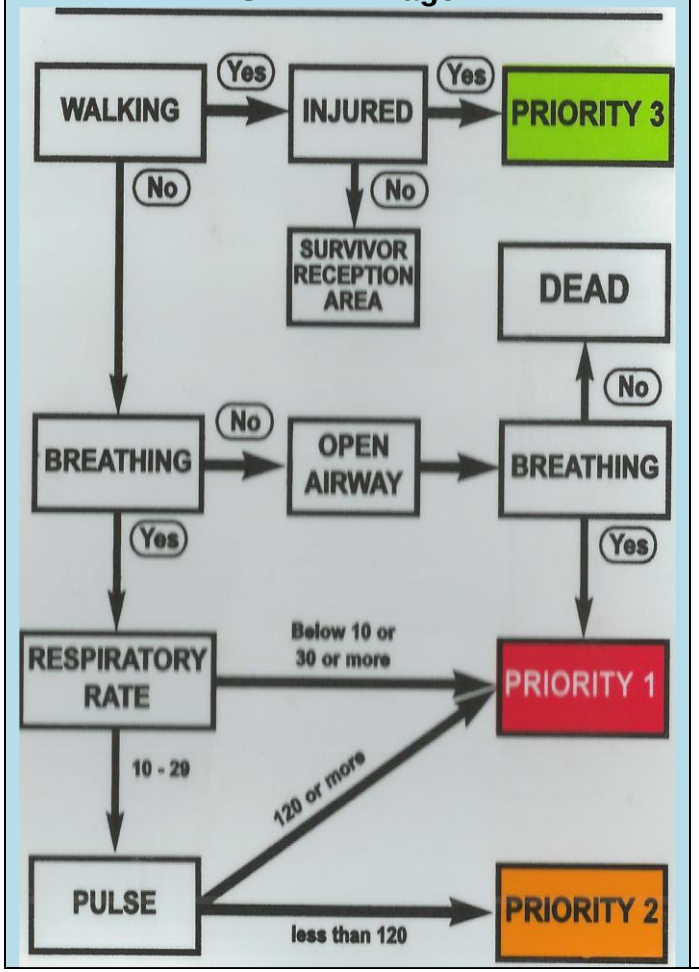

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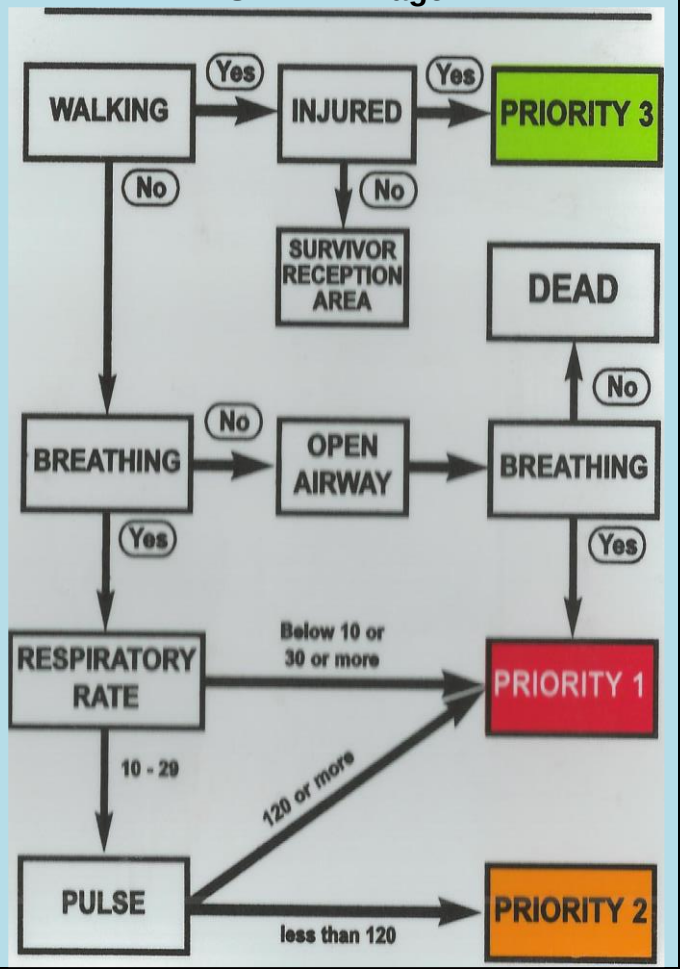

SMART TRIAGE

This decision-making system is used to assign each casualty with a priority = **ONE TWO THREE**

- Stop to give only basic, life-saving treatments, following M.A.R.C.H.E.
  - Stop all major bleeds
  - Ensure an open airway
  - Are they breathing?
  - Do they have a pulse?
- Initial triage and treatment should take no longer than 30-60 seconds for each casualty
- Anyone who can walk at the scene will be tagged **GREEN / Priority 3**
- Every unconscious casualty will be placed in recovery position. Further treatment will only be done once all casualties have been triaged

- If / when possible, move casualties into groups by priority
- Casualties **deteriorate.** CCTs can + should increase a priority if needed. CCTs cannot lower a priority, or pronounce a casualty dead

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M.A.R.C.H.E

VERY quick initial check to determine priority. It is **NOT** a full vitals check. Do one step at a time. Do **NOT** move to the next step until the current step has been addressed

- **M – MAJOR BLEEDING**
  - Stop all major, life threatening bleeds with
    - Direct pressure
    - Indirect pressure
    - Tourniquet
- **A – AIRWAY**
  - Is there an airway? Reposition if needed
  - Manual C-spine control if needed
- **R – RESPIRATION**
  - Check for breathing. Give 2 rescue breaths if not breathing
  - Is it fast / slow? Shallow / deep?
- **C – CIRCULATION**
  - Check for pulse. Start CPR if no pulse
  - Is it fast / slow? Weak / strong?
- **H – HYPOTHERMIA / HEAD**
  - Remove wet clothing
  - Cover casualty to prevent heat loss
  - C-collar + check for head injuries
- **E – EVERYTHING ELSE**
  - Head to toe - expose + examine
  - Monitor vitals + document findings

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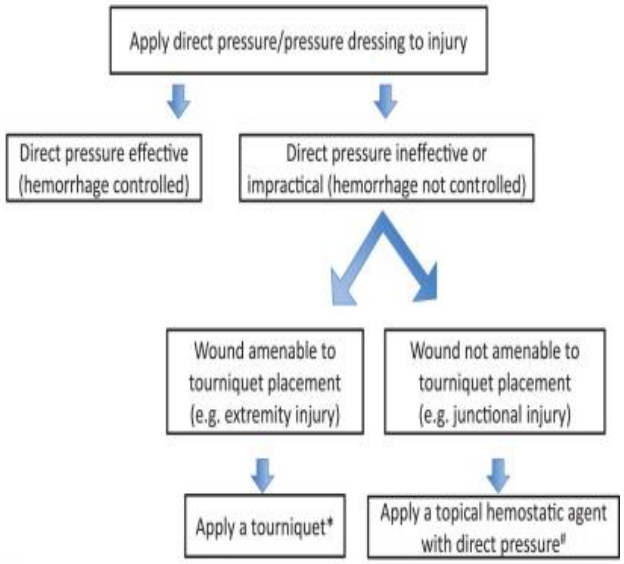
HEMMORRHAGE CONTROL



- Do not remove dressing if soaked through. Apply 2nd dressing directly overtop
- Do not remove / adjust / loosen tourniquet. If ineffective, apply 2nd tourniquet proximally
- Do not remove imbedded objects. Secure objects in place, pad + dress around objects
- Elevate bleeding limb to reduce blood flow

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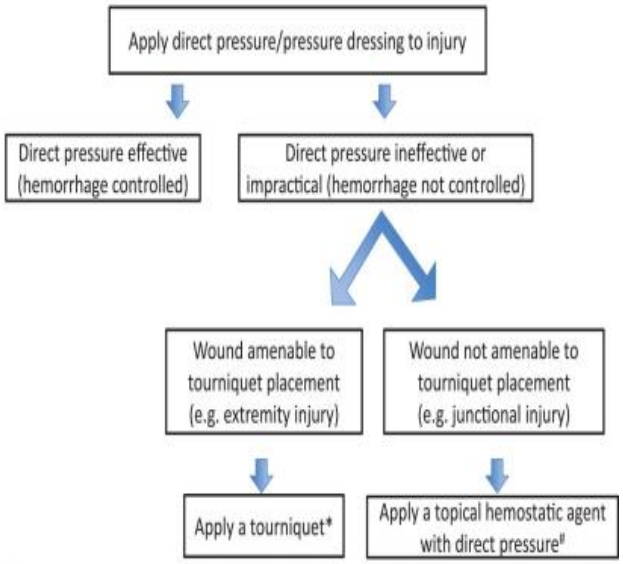

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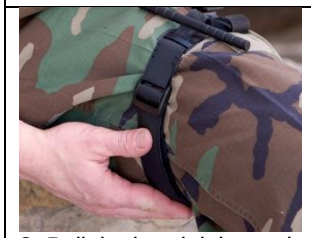
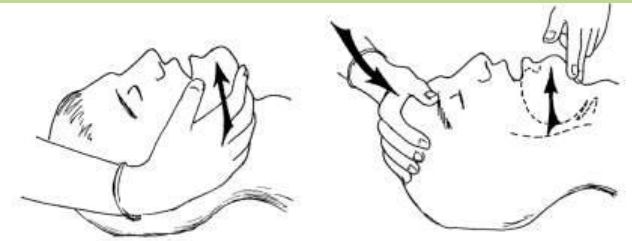
1. Pass band through the inside slit of buckle	**2.**Pass band through the outside slit of buckle		**1.** Pass band through the inside slit of buckle	**2.**Pass band through the outside slit of buckle
 **3.** Pull the band tight, and securely fasten the band back on itself.	**4.**Twist the windlass until bright red bleeding has stopped		**3.** Pull the band tight, and securely fasten the band back on itself.	**4.**Twist the windlass until bright red bleeding has stopped
5. Lock the windlass in place in the clip. Secure with velcro strap	**6.** Document what time the tourniquet was applied, and its location		**5.** Lock the windlass in place in the clip. Secure with velcro strap	**6.** Document what time the tourniquet was applied, and its location

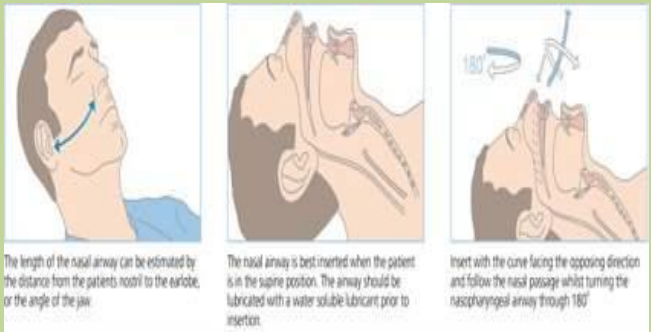
AIRWAY

An unconscious pt cannot keep their own airway open



Jaw Thrust - spinal injury Chin lift - no spinal injury

Nasal Pharyngeal Airway Insertion



Contra-indications for NPA

- Active nose bleeds
- Known or suspected nasal fractures
- Known or suspected basal skull fractures

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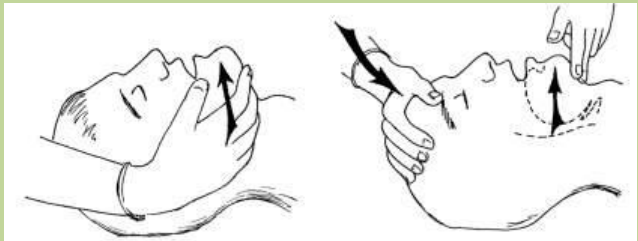

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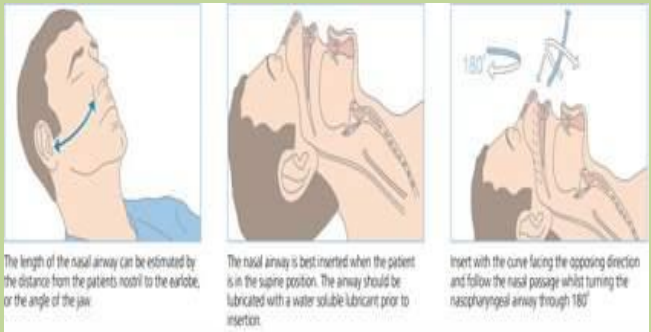

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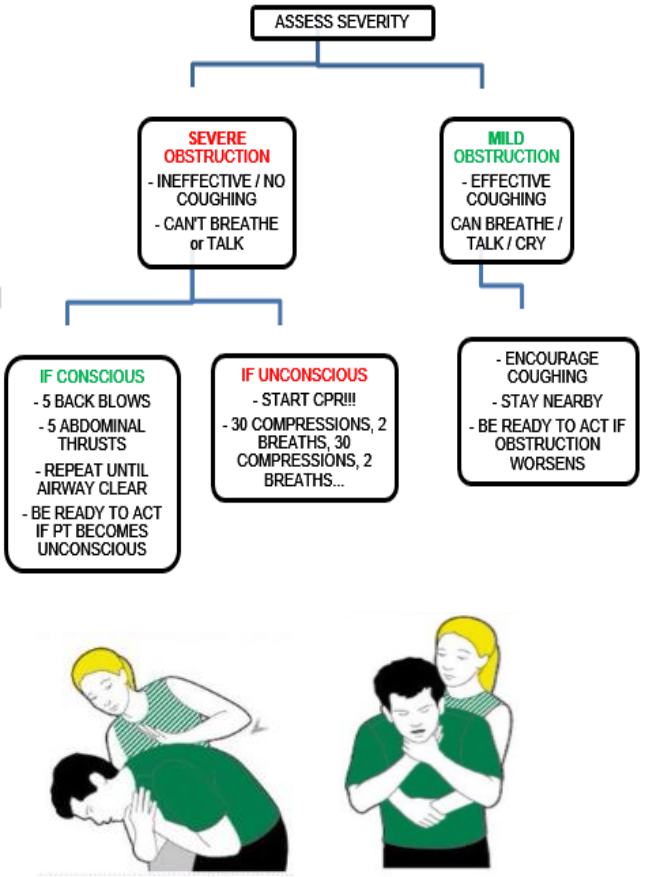

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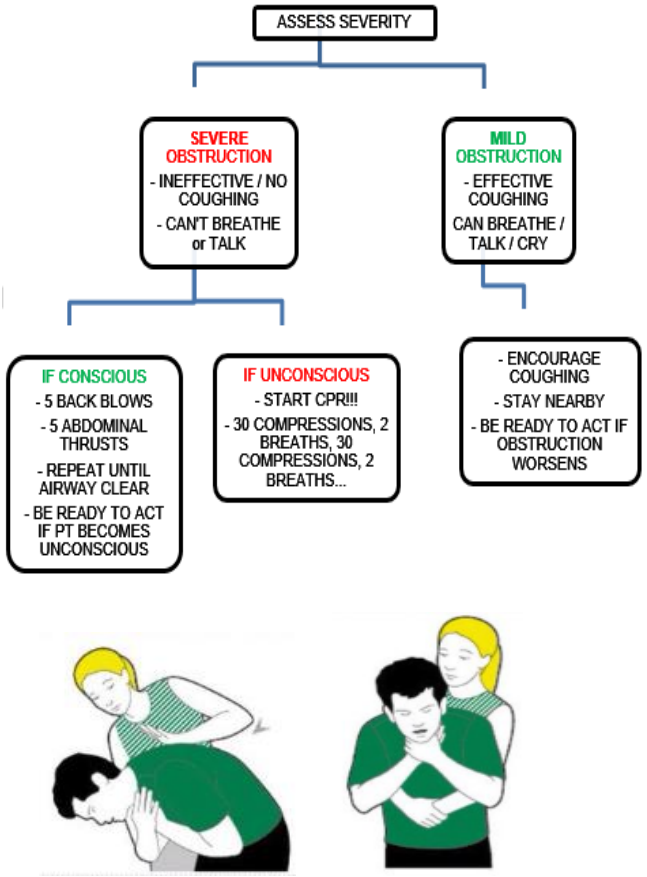

CPR + AED

Recognize cardiac arrest + **Activate** emergency response system **ASAP!!!**

Check pulse
As soon as you find that there is no pulse, START CPR!!! Start with chest compressions = 30 compressions, 2 breaths, 30 compressions, 2 breaths…

CALL FOR HELP + GET AED RIGHT AWAY!

If pulse is present but **breathing is absent**, open airway and give rescue breathing. Give **1 breath every 5 - 6 seconds**

Good CPR is:

- chest compressions of adequate rate
  - at least 100 / minute
- chest compressions of adequate depth
  - adults: at least 2”
  - infants + children: at least 1/3 chest depth. Infants = 1 ½” Children = 2”
- allow complete chest recoil after each compression
- minimize interruptions in compressions
- avoid excessive ventilation, don’t overinflate lungs

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BURNS

Stop the Burning Process
- Remove pt from source of heat / burn
- Brush off powder chemicals

- Flush with cool tap water until burning stops

- Remove clothing + jewelry from burned areas

Assess the Burn
- Check burn for % body surface area + severity
- Automatic Priority 1 if burns to face, neck, groin

Cover the Burn
- Use dry burn dressings, secured lightly with bandages
- DO NOT APPLY CREAMS!

Prevent Hypothermia
- Cover the pt with blankets to prevent heat loss

High Flow Oxygen
- If signs of inhalation, give high flow oxygen

Intravenous Fluids
- Consult with PA/Med Tech and prepare equipment as required

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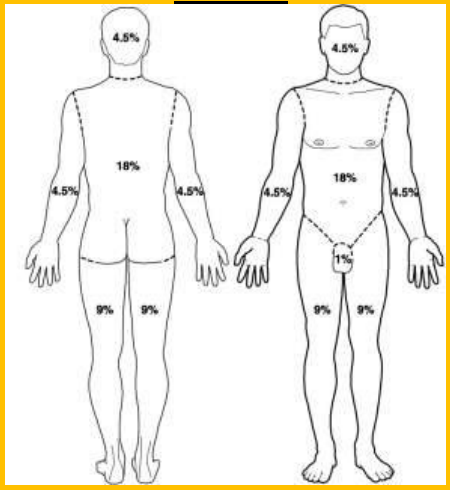

Depth of burn	Characteristics	Example
First degree burn	• Redness • Painful	• Sunburn
Second degree (Partial thickness)	• Red or mottled • Blistered • Very painful	• Contact with hot liquids
Third degree (Full Thickness)	• Dark and leathery • Dry • Not that painful	• Fire • Electricity or lightning • Prolonged exposure to hot liquids/objects

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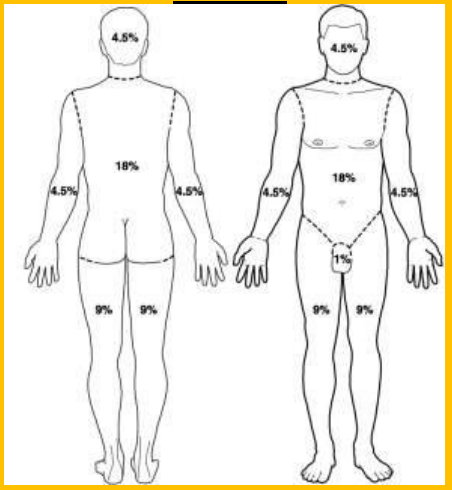

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VITAL SIGNS

Level Of Consciousness - AVPU

- Alert: responding normally
- Verbal: only responds to voice
- Pain: only responds to pain
- Unresponsive: no response

RESP RATE

- Look, listen, feel for 15 sec X 4
- Rate, Rhythm, Depth

HEART RATE

- Feel Radial or Carotid for 15 sec X 4
- Rate, Rhythm, Strength

BLOOD PRESSURE

- If kit available
- If radial pulse present, BP = +/- 80

O2 SAT

- Use finger on uninjured side

SKIN

- Colour, Temperature, Texture

CORE TEMPERATURE

- Especially for MOB / hypothermia

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M.I.S.T.

Use M.I.S.T. to report on casualty status, and to document your findings

M – MECHANISM OF INJURY

- What happened?

I – INJURIES

- What are the injuries?

S – SIGNS & SYMPTOMS

- **Signs** - blisters, bleeding, etc
- **Symptoms** – pain, dizziness, etc

- LOC: AVPU
  - Alert
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- **Vital Signs**

T – TREATMENT

- What have you done for the patient?

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PATIENT HISTORY

Gather some info from your patient, or any witnesses

S – SIGNS AND SYMPTOMS

- What hurts? Where does it hurt?

A – ALLERGIES

- Do you have any allergies?

M – MEDICATION

- Do you take any medication?
- Did you take medication today?

P – PAST MEDICAL HISTORY

- Do you have any medical conditions?

L – LAST MEAL

- When was the last time you ate?
- What did you eat?

E – EVENTS LEADING TO THE INCIDENT

- What were you doing before you got hurt?

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PATIENT HISTORY cont.

O – ONSET
- When did it start?
- Sudden or gradual

P – PROVOCATION
- What triggers the pain?

Q – QUALITY
- Describe the pain - crushing, stabbing, burning, sharp, etc

R – RADIATION
- Does the pain spread / travel?

S – SEVERITY
- Level of pain from 0 - 10
- Keep in context of the pt's 10 / 10

T – TIME
- When did it start?
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COMPLETE BODY SURVEY

Take time to thoroughly check every area + system

DCAPP: deformity, contusion, abrasion, puncture, paradoxical chest movement
BLS: bruising, laceration, swelling
TICS: tenderness, instability, crepitus, subcutaneous emphysema

- **HEAD + NECK**
  - Check skull, face, neck for DCAPP BLS TICS
  - Check pupils for PERLA
  - Look for battle signs + fractures
  - Check mouth + teeth
  - Vein distention
  - Midline trachea
- **CHEST + RESP**
  - Check chest for DCAPP BLS TICS
  - Look for paradoxical movement + subcutaneous emphysema
  - Listen to both lungs, if kit available
- **ABDOMEN + BACK**
  - Check abdo for DCAPP BLS TICS
  - Check back for DCAPP BLS TICS, use proper roll if spinal concerns
- **PELVIS + GROIN**
  - Check pelvis for DCAPP BLS TICS
  - Check for loss of bowel / bladder function
- **LEGS + FEET**
  - Check legs + feet for DCAPP BLS TICS
  - Check cap refill on both feet
  - Check for motor function + sensation
- **ARMS + HANDS**
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ONGOING CASUALTY CARE - Place casualties in triage areas by priority - Report any significant changes in vitals / treatments to higher medical / Senior Casualty Clearer - Unstable patients = check vitals every 5 min - Stable patients = check vitals every 15 min - Reassess MARCHE every time you check vitals. Are all treatments still effective? - Reassure your patients. Talk to them, tell them what you are doing + what is going on - Keep your patients warm + comfortable - Document your findings for each patient • treatments + what time they were given • vitals + what time they were taken • all other info you have for each patient - Keep each documentation with each patient		**ONGOING CASUALTY CARE** - Place casualties in triage areas by priority - Report any significant changes in vitals / treatments to higher medical / Senior Casualty Clearer - Unstable patients = check vitals every 5 min - Stable patients = check vitals every 15 min - Reassess MARCHE every time you check vitals. Are all treatments still effective? - Reassure your patients. Talk to them, tell them what you are doing + what is going on - Keep your patients warm + comfortable - Document your findings for each patient • treatments + what time they were given • vitals + what time they were taken • all other info you have for each patient - Keep each documentation with each patient

LIMB INJURIES+ SPLINTING

- Expose injured limbs + assess DCAPP BLS TICS
- Immobilize injuries w Sam splints, triangulars, tensors
- Before + After immobilizing, check pulses / cap refill + check sensation below the injury
- Compare injured to uninjured side for swelling, deformities, pulses, sensation
- Watch for signs of shock, especially w large bone fractures
- Be aware of the tactical situation + be prepared to transport your pt safely if they can't walk

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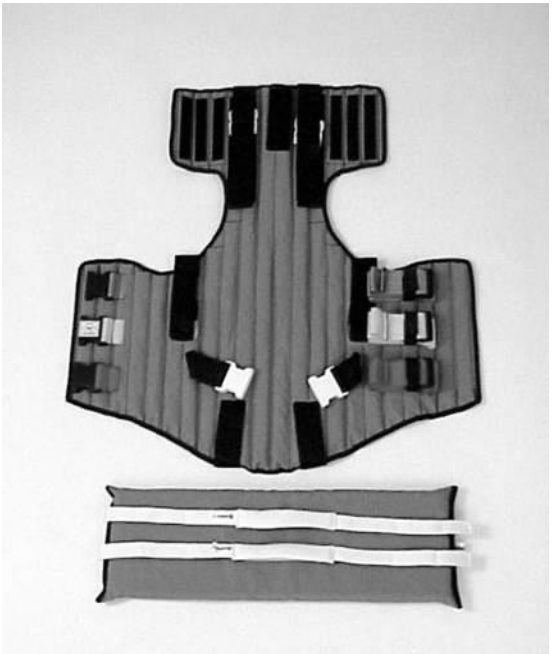

Put straps on in this order "*My Baby Looks Hot Tonight*"
Tighten straps in same order

My	-	middle strap
Baby	-	bottom strap
Looks	-	leg straps
Hot	-	head straps
Tonight	-	top strap

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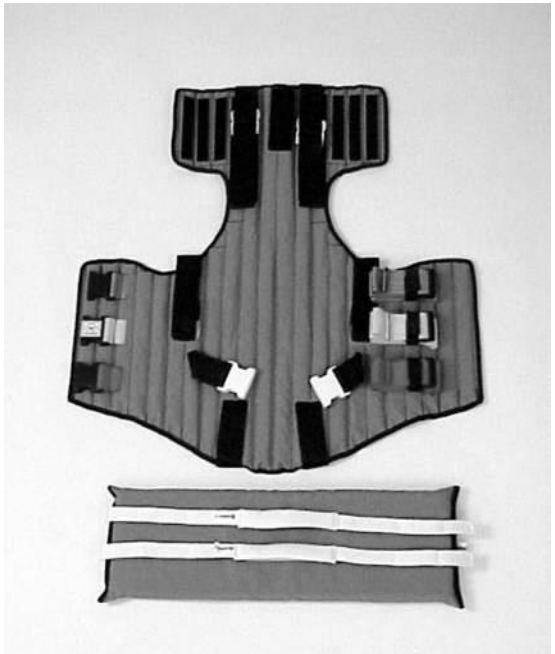

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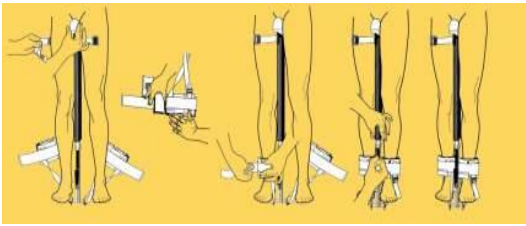
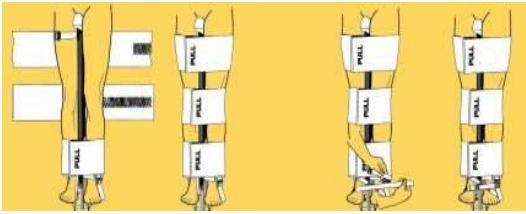
SAGER TRACTION SPLINT





- Traction = 10% of pt body weight up to 15 lbs max
- Closed fractures only, not near joints
- Before + After application of Sager splint, check for absence or presence of distal pulses + check for absence or presence of sensation

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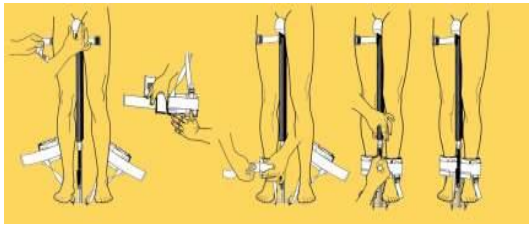

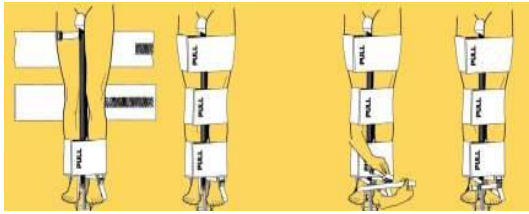

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TROUBLESHOOTING

- Treat your entire casualty. Don't get tunnel vision and focus on just one injury
- Reassess vitals + all treatments every time your casualty is moved. Did anything change? Are the treatments still effective?
- Get vitals manually. You will get much more info on your pt's status through touch. Pulse, resp, skin temp, swelling, shivering, etc all need hands-on
- If your casualty is deteriorating, you may have missed something. Go back to the beginning of MARCHE, then do another complete body survey
- O2 Sat monitors will NOT be accurate w Carbon Monoxide poisoning + hypothermia. Don't rely on the machine, treat the pt's signs + symptoms
- Don't go beyond your training w treatment / equipment / medical advice. CCTs are expected to treat patients + stabilize injuries until higher medical care is available
- Never abandon your casualty. If you must leave the scene, hand over care to a capable bystander until you return
- Ask for help! Ask other CCTs for advice. Ask manning pool to carry equipment. Use you resources
- Talk to your casualty as you care for them. Tell them what you're doing + why. This will help you stay focused + will reassure your patient
- Do SOMETHING! Identify a medical problem + take actions to solve it. Use common sense + do your best to care for your shipmates

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Abbreviations and short-hand terms for Documentation

	DESCRIPTION
NPA	Nasopharyngeal Airway. Flexible airway, inserted into the nostril
OPA	Oropharyngeal Airway. Rigid plastic airway, inserted into the mouth
L/Min	Litres Per Minute. Flow rate of O2 from tank
O2 Sat or SPO2	Saturation Point of O2. The percentage of O2 in the blood
CO	Carbon Monoxide. Percentage of CO in blood
SOB	Short of Breath. Wheeze, struggling, gasping
Cap Refill	Capillary Refill. Blood return to nail bed or skin after pressing
BP	Blood Pressure. Manually or by monitor
HR	Heart Rate. Beats per minute
RR/Resp	Respiratory Rate. Breaths per minute
Pt	Patient. The casualty
L / R	Left / Right. Referring to arms, legs, etc
Bilat	Bilaterally. Both sides, arms, legs, etc
M / F	Male / Female
YO	Years Old. Your pt's age
Tx / Tmt	Treatments given
Fx	Fracture
VSA	Vital Signs Absent. No detectable pulse or resp, pt is unresponsive
PERLA	Pupils Equal, React to Light, Accommodating. Pupils reacting normally
TBSA or BSA	Total Body Surface Area. Percentage of the pt's body that is burned
N / V / D	Nausea, Vomitting, Diarrhea
Drsg	Dressing. A bandage or dressing applied
H / A	Headache
NKA NKDA	No Known Allergies, No Known Drug Allergies
ETOH	Alcohol. Referring to intoxication

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